

The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

Global Data · 2023 Estimates | IHME Global Burden of Disease

Source: Our World in Data — Mental Health
Saloni Dattani, Lucas Rodés-Guirao, Hannah Ritchie, and Max Roser (2023)

For: Public Health Policy Analysts & Mental Health Program Planners

Hundreds of Millions Affected

1 in 3 Women and 1 in 5 Men Face Major Depression in Their Lifetime

1 in 3

WOMEN

will experience major
depression in their lifetime

1 in 5

MEN

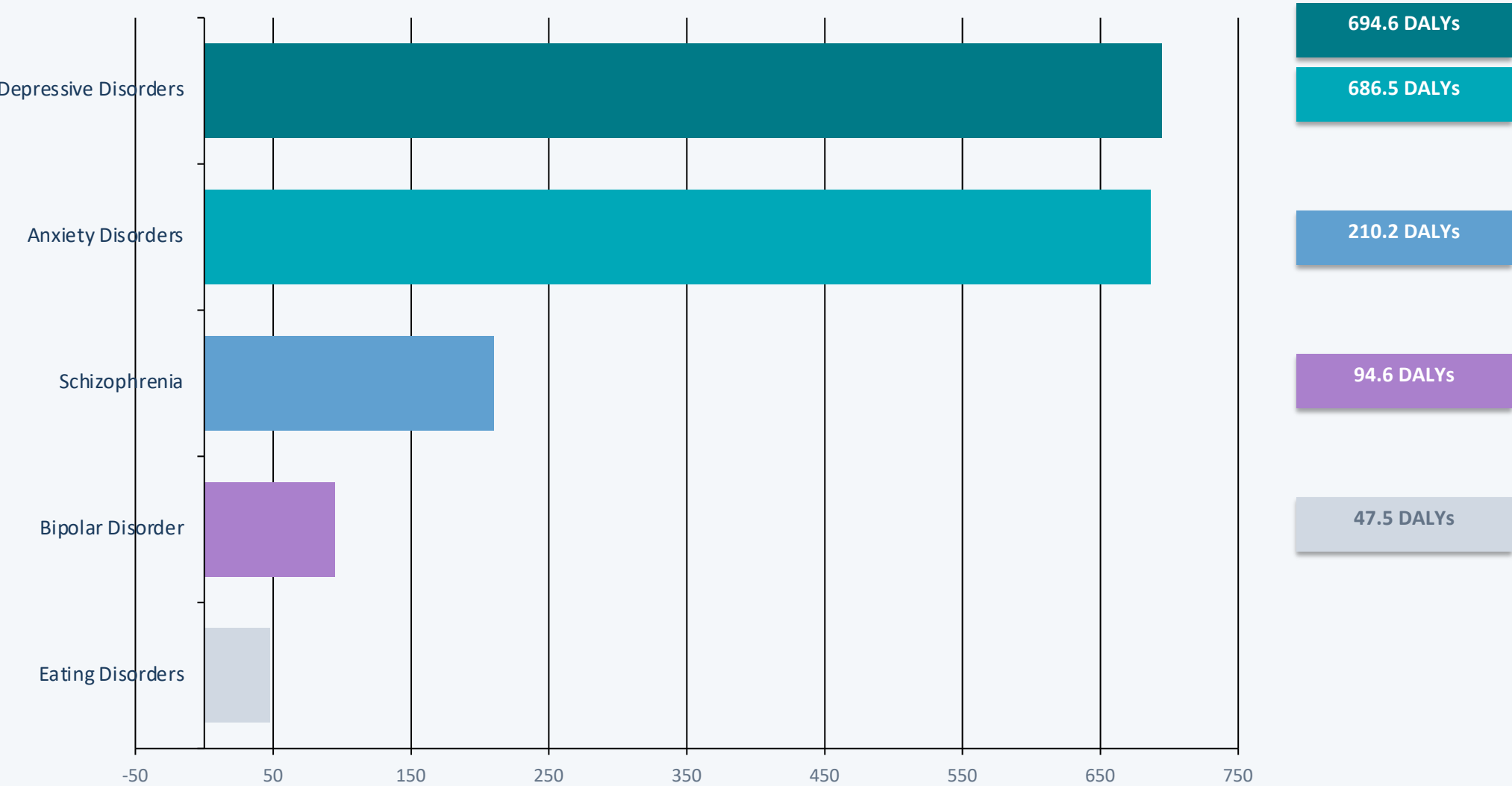
will experience major
depression in their lifetime

Why This Matters for Policy

- Hundreds of millions affected globally each year
- Depression & anxiety: most common, highly treatable
- Schizophrenia & bipolar: rarer but cause severe disability
- Mental illness impairs work, relationships, and well-being
- Underdiagnosis means the true burden is likely higher

Depression and Anxiety Dominate Global Mental Health Burden

DALYs per 100,000 people · World · 2023

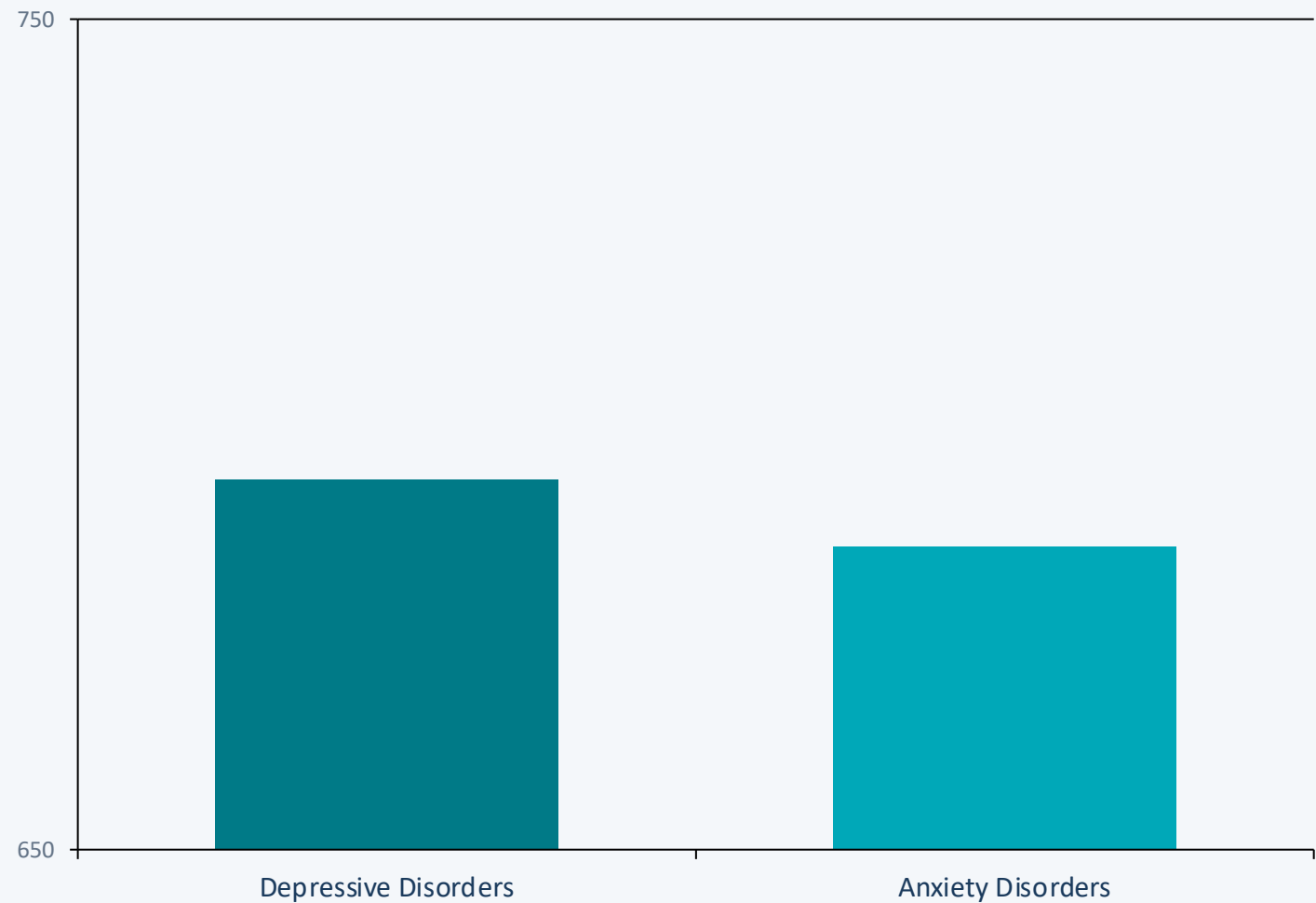


DALY = Disability-Adjusted Life Year (years of healthy life lost)

Source: Our World in Data / IHME, Global Burden of Disease (2023)

Depression and Anxiety Carry Nearly Equal Disease Burden Worldwide

Side-by-side DALYs per 100,000 · 2023



Key Insight

694.6 DALYs
Depressive Disorders

686.5 DALYs
Anxiety Disorders

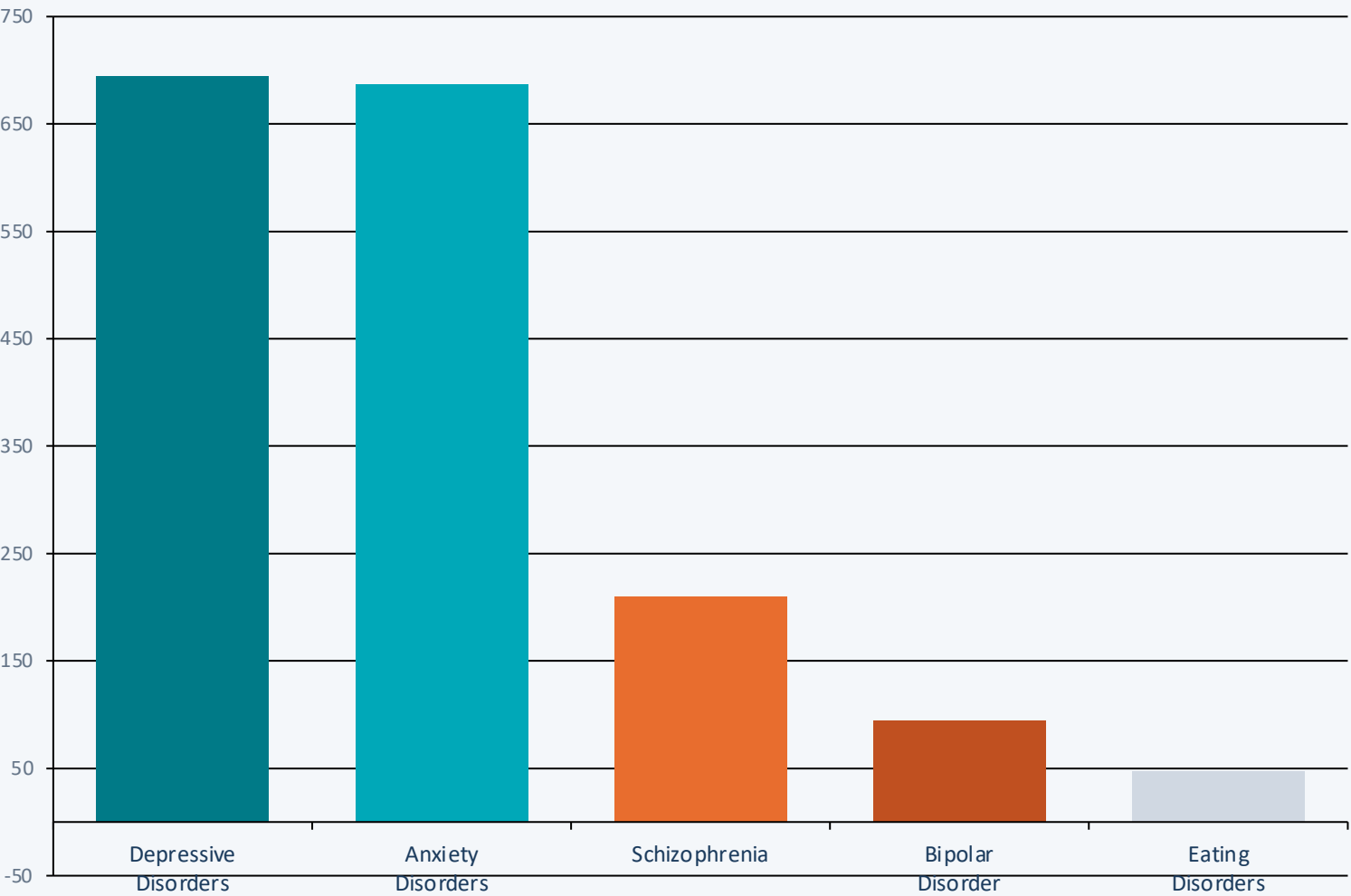
~1.4% difference — statistically near-identical burden

Together: ~80% of total mental health DALYs across the 5 categories shown

Both are amenable to evidence-based treatment — yet millions go untreated

Schizophrenia and Bipolar Disorder: Lower Prevalence, Severe Individual Impact

Contrasting population-level burden vs. individual severity



Schizophrenia

**210.2 DALYs
per 100,000**

Affects fewer people but causes profound disability—often requiring lifelong specialist care.

Bipolar Disorder

**94.6 DALYs
per 100,000**

Episodes of mania and depression create high individual burden despite lower prevalence.

Policy note: While depression/anxiety dominate at population level, specialist care for schizophrenia and bipolar must be sustained — the individual stakes are very high.

Source: Our World in Data / IHME, Global Burden of Disease (2023)

Treatment Is Available but Often Lacking or Poor Quality

The persistent gap between need and care



STIGMA

Many people are uncomfortable sharing mental health symptoms with healthcare professionals or those they know.



ACCESS

Treatment is often lacking — especially in low- and middle-income countries where resources are scarce.



QUALITY

Even where treatment exists, it may be of poor quality or not aligned with evidence-based guidelines.



UNDER-REPORTING

Discomfort with disclosure makes it hard to estimate true prevalence — the actual burden is likely higher than reported.

Mental illnesses ARE treatable — yet millions go without adequate care. Closing this gap is a core policy imperative.

"Good data is essential to understand these conditions — how, when, and why they occur, how many people are affected, and how they can be treated effectively and safely."

How People Cope: From Medication to Faith to Family Conversations

Global survey data reveals wide variation in coping strategies for anxiety and depression



PRESCRIBED MEDICATION

- Common in higher-income settings
- Access tied to healthcare system capacity
- Includes antidepressants and anxiolytics
- Requires trained prescribers & supply chains



RELIGIOUS & SPIRITUAL ACTIVITIES

- Prevalent in many regions worldwide
- Often the primary or only source of support
- Reflects deep cultural and community bonds
- Important to engage faith communities in MH policy



TALKING TO FRIENDS OR FAMILY

- Widely reported across all regions
- Low-cost, accessible form of support
- Stigma can limit openness even with family
- Peer support programs can formalize this resource

Policy implication: Responses vary widely across cultures and economies. Effective interventions must be culturally sensitive — a one-size-fits-all approach will not close the treatment gap.

Key Takeaways

Prioritizing Depression, Anxiety, and Closing the Treatment Gap

01

Depression & Anxiety Dominate

Together generating ~694 and ~687 DALYs per 100,000 respectively, they represent ~80% of the mental health disease burden across the five categories studied. Scaling evidence-based interventions for these disorders should be the top policy priority.

02

Severe Disorders Carry Disproportionate Individual Burden

Schizophrenia (210.2 DALYs) and bipolar disorder (94.6 DALYs) affect fewer people but cause severe disability. Specialized care must be sustained and not crowded out by the focus on common disorders.

03

Treatment Exists — but Access and Quality Are Inadequate

Mental illnesses are treatable, yet significant gaps persist, especially in low- and middle-income countries. Investment in care infrastructure, training, and supply chains is essential.

04

Stigma Suppresses Help-Seeking and Distorts Data

Discomfort with disclosure means true prevalence is likely higher than reported. Anti-stigma campaigns and anonymous access pathways are critical complements to clinical services.

05

Culturally Sensitive Policy Is Non-Negotiable

Coping strategies span medication, spiritual support, and social networks. Effective programs must engage communities and adapt to local contexts rather than imposing uniform solutions.